Certify

PC-1	What health management or care navigation apps would help you understand and manage your (or your loved ones) health needs, as well as the actions you should take?	Care navigation tools can be highly effective in helping patients manage complex care journeys. These tools often do a good job of surfacing clinical services, coordinating care teams, and guiding patients through diagnoses and treatment plans. But while they provide clinical context, they often lack one of the most critical pieces of information patients need to take action: whether a provider is actually covered by their insurance plan. While this idea may not be novel, without accurate, real-time provider directory data, the tools fall short. Patients may be guided to the right type of care, but the wrong provider. They may be shown availability, but not know whether the provider is in-network. This can lead to confusion, unnecessary costs, and/or delays in care. This is the gap Certify is built to fill. Certify provides the infrastructure needed to connect health system-based navigation tools to trusted, continuously updated payer directory data. Our platform can serve as a national source of truth for provider participation, affiliations, locations, and specialties, accessible through open APIs that can power consumer-facing tools in real time. When care navigation apps are connected to Certify's infrastructure, they gain the ability to: Surface only in-network providers for a patient's specific insurance plan, avoiding out of network claims Reflect accurate practice locations and active affiliations Reduce misrouting, mispayment and administrative friction Empower patients to act with confidence—because the information is correct The result is a dramatically more useful experience for patients and caregivers, and the potential to reduce billions

		of wasted healthcare dollars, that are wasted resolving disputes that were caused by inaccurate provider data. Clinical guidance is essential, but it becomes actionable only when paired with accurate, real-time network and provider data. Certify makes that possible, helping these tools reach their full potential and improving access, equity, and patient outcomes along the way.
PC-4	What features are missing from apps you use or that you are aware of today?	A commonly overlooked feature in today's health apps—especially patient-facing scheduling and navigation tools—is the ability to see details of what health plans providers are in network with, and the corresponding patient financial responsibility. Patients may be able to search for providers and schedule appointments in a self service capacity, but without integration into provider directories, ***or even with integration into innacurate provider directories***, downstream friction, surprise billing, missed opportunities for care, and general waste will continue to plague payers, providers, and patients alike. With Certify in place as the national provider directory, this becomes a solvable problem. Our infrastructure allows patient-facing care navigation and scheduling apps to plug into a real-time, verified source of truth about provider network participation, by group, location, and specialty, including details of patient financial responsibility based on the type of service being pursued by the patient. That unlocks the ability for apps to finally deliver the kind of experience patients expect in every other part of their lives—searching for care based on: Specialty and clinical focus Insurance network participation and covered benefits Location and practice site Availability that matches the patient's preferred timeframe Language, cultural background, or other personal preferences By integrating Certify's provider data layer with provider scheduling systems, digital health apps could show only providers who are truly available, in-network, and relevant for the patient's needs. This creates a seamless experience where patients can not only find the right provider but also take action immediately, with confidence that the information is accurate, and that there won't be any

		surprise bills. In short, what’s missing today is an accurate provider data infrastructure empowering actionable clarity. Certify provides the infrastructure to enable it, making digital tools more useful, more trusted, and more effective in helping patients access appropriate care, while reducing downstream waste.
a.	a. What apps should exist but do not yet? Why do you believe they do not exist yet?	From Certify's perspective, the problem is not that necessary apps are missing, it's that the infrastructure needed to power certain functionality within existing apps is missing. One missing piece from our perspective, as previously described, is patient visibility into complete and accurate financial responsibility details related to providers participation in health plan networks, by specialty, by group, and by location, at the time of scheduling. Layering in accurate provider data into the patient scheduling process would make navigating the health care system easier for patients, reduce surprise billing and out of network claims, and save patients, payers, and providers time dealing with the clean up and cost that comes along with patients accidentally seeing an out of network provider.
PC-5.	What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?	Regardless of whether or not a national directory is in place, Certify's stance is that providers should be incentivized in APMs to keep their demographic data up to date across all plans they are in network with. Currently, without a single national provider directory in existence, updates may need to be made across dozens of different platforms for a single provider, placing a significant burden on providers and/or their administrators. This is why many providers and health systems aren't as proactive in updating their demographic information as they should be. Having a single provider directory would greatly ease this burden and should naturally increase provider proactivity in keeping their information up to date, but providing additional incentives would surely drive participation even further. Given the disruption and hidden costs inaccurate provider data causes across the healthcare ecosystem, Certify believes adding incentives is low hanging fruit to lowering total cost of care, and improving the health care experience.
PC-8, a.	What data is valuable, but hard for patients and caregivers, or app developers and other technical vendors, to access for appropriate and valuable use (for	Provider data is available - but it's available in too many different places, is discrepancy laden, and usually lacks the level of contract related details that are needed to help patients make accurate decisions about where they should access care. With Certify in place as the infrastructure that feeds the national directory, powered by AI driven entity

	example, claims data, clinical data, encounter notes, operative reports, appointment schedules, prices)?	resolution and survivorship logic, those issues that make provider data limited in its value to patients, go away, saving patients, providers, and plans time, and reducing costs across the industry.
b.	What are specific sources, other than claims and clinical data, that would be of highest value, and why?	As previously stated, the value of provider data - including accurate directory data, primary source data, and self reported data - and the impact it has across healthcare is often overlooked. Without easy access to these data sources, patients see out of network providers, resulting in out of network claims, claims are rejected because services are rendered at out of network locations, or by the wrong specialty type, and payments can be made to incorrect providers or addresses. The provider network infrastructure that Certify has built, and the opportunities it opens up for a national provider directory, does away with those aforementioned access issues, and downstream ramifications. Our platform collects, attributes, and unifies data from hundreds of primary sources, provider self-attestations, third-party partners, payer networks, and delegated vendors, making access to a real-time, AI-driven platform possible. Unlike other platforms in the industry, our foundational data first, open API approach, can offer a source of truth for provider participation, affiliations, locations, and specialties, that can be trusted, and made available to other technology in the healthcare ecosystem, reducing friction across the healthcare lifecycle for all stakeholders. The result is a dramatically more useful experience for patients and caregivers, and the potential to reduce billions of wasted healthcare dollars, that are wasted resolving disputes that were caused by inaccurate provider data. Clinical guidance is essential, but it becomes actionable only when paired with accurate, real-time network and provider data. Certify makes that possible, helping these tools reach their full potential and improving access, equity, and patient outcomes along the way.
PC-12	What are the most valuable operational health data use cases for patients and caregivers that, if addressed, would create more efficient care navigation or eliminate barriers to competition among providers or both?	Certify views this question from the perspective of inaccurate provider data, and the impact it can have on patients as they seek the right provider to meet their health needs, and the corresponding downstream impact when they receive care out of network, or from a specialty that isn't covered. Layering accurate provider data into care navigation tools, which can be achieved with Certify in place as the infrastructure powering a national provider directory, would help patients make sure they are receiving culturally competent care, from a provider that is in their network and providing a covered benefit, with upfront visibility to their financial responsibility. The ultimate result

		is a better pre-encounter patient experience, and the potential to save billions of wasted healthcare dollars that are spent on post-encounter clean up.
PR-1	What can CMS and its partners do to encourage providers, including those in rural areas, to leverage approved (see description in PC-5) digital health products for their patients?	Implementing an infrastructure to support a national provider directory, that feeds data into other patient and provider facing apps, would naturally help encourage providers to be more proactive in updating their demographic information through a reduction of required locations to do so. Additionally, integrating provider data that ultimately surfaces patient financial responsibility at the time fo scheduling, would improve the front end health system navigation experience for patients. Furthermore, it's Certify's belief that providers should be incentivized in APMs to keep their information accurate and up to date, given they are the ultimate source of truth, and the broad reaching impact inaccurate data has on the health care experience.
PA - 4	What would be the value to payers of a nationwide provider directory that included FHIR end points and used digital identity credentials?	The value to payers of a nationwide provider directory built on FHIR standards and supported by digital identity credentials is enormous, particularly if Certify's infrastructure is in place to power it. With Certify as the backbone, individual health plans would be able to access and contribute to a continuously verified national provider directory via open APIs. This shared infrastructure would eliminate the need for each plan to maintain its own fragmented, siloed system, and would replace costly, redundant workflows with a real-time, trusted source of provider truth. Key benefits to payers include: Lower administrative costs: Payers today invest significant resources in collecting, verifying, and maintaining provider data, both for compliance and operational use. By plugging into Certify's infrastructure, these efforts are streamlined, and duplicative verification processes are reduced or eliminated entirely. Reduction in out-of-network claims: Inaccurate provider directories are a major driver of unintended out-of-network care. With a real-time national directory that accurately reflects plan participation at the location and provider level, patients are routed correctly, and costly surprises are avoided. Fewer rejected claims and denials: Many claim rejections

		stem from simple mismatches in provider data, such as an outdated TIN, NPI, or practice affiliation. Certify ensures all this data is up to date and consistent across the payer ecosystem, reducing friction and improving first-pass claim acceptance. Reduction in downstream waste: Clean provider data upstream means fewer issues downstream, including fewer member grievances, less time spent on appeals, reduced call center volume, and better member satisfaction overall. When layered with a digital identity credentialing system, this infrastructure becomes even more powerful. Providers can verify their own information through authenticated channels, ensuring that the data submitted is accurate, attributed, and audit-ready. This significantly reduces the risk of fraud, enhances trust in the data, and provides payers with a much clearer chain of custody for every update. In short, the combination of Certify's real-time infrastructure, accessible via APIs, and a verified digital identity credentialing layer creates a scalable, accurate, and secure foundation for managing provider data nationally. For payers, this means reduced costs, improved operational efficiency, better compliance, and a more trusted experience for both providers and members.

TD-5	How could a nationwide provider directory of FHIR endpoints improve access to health information for patients, providers, and payers? Who should publish such a directory, and should users bear a cost?	A nationwide provider directory of FHIR endpoints would significantly improve access to health information across the healthcare ecosystem, but only if built on a real-time, scalable infrastructure like Certify's. With Certify in place, this directory would not just list providers and endpoints, it would provide verified, continuously updated, and interoperable provider data that powers smarter decision-making, better navigation, and more efficient operations across patients, providers, and payers. For patients, this infrastructure would create a dramatically more transparent and navigable healthcare experience. Care navigation and scheduling vendors could layer their tools directly on top of Certify's infrastructure, enabling patients to see exactly which providers are in-network, at which locations they practice, and under which group or facility affiliation. Beyond provider matching, this infrastructure could also support visibility into what services are covered and what the patient's out-of-pocket responsibility will be. The result is a meaningful reduction in the confusion, billing surprises, and post-service financial stress that often undermines patient trust and satisfaction. For providers, a unified national directory would simplify a frustrating and fragmented administrative process. Today, providers must update their demographic and participation information across dozens of separate directories. With Certify's infrastructure in place, updates made once through a validated source could cascade across all connected systems, reducing the number of touchpoints and improving data accuracy across the board. This also reduces the need to respond to redundant payer outreach or rework claims tied to outdated or incorrect data. For payers, the value is equally compelling. Plugging into Certify's infrastructure via API allows payers to access a continuously verified source of provider data tied to open API endpoints. This reduces the overhead required to manage internal directories, improves accuracy across claims and member-facing tools, and decreases waste caused by out-of-network claim errors or data mismatches. As noted in our previous response, fewer denied claims, better routing, and reduced fraud risk all translate to measurable operational and financial gains. Governance and Access The directory itself should be governed through a public-private collaboration. CMS or ONC could define the

		data standards, interoperability rules, and governance principles, while infrastructure and directory operations should be managed by a technically capable partner. Certify is well positioned to serve in this role, already managing real-time provider data with robust lineage, survivorship logic, and cross-stakeholder configurability. Cost Structure Basic access—particularly for patients, non-profit entities, and regulatory use cases—should be free and open. For commercial users, high-volume integrations, or value-added services, a tiered pricing model tied to actual system cost drivers would support the sustainability of the infrastructure while still prioritizing equitable access. Importantly, the operational cost of maintaining this national directory would be more than offset by the cost savings it enables—through reduced administrative overhead, fewer denied or misrouted claims, lower fraud risk, and improved patient and provider satisfaction. In this way, the directory would ultimately pay for itself. In short, a nationwide directory of FHIR endpoints, built on Certify's infrastructure, would not only improve access to health information but also reduce administrative friction, eliminate duplication, and increase trust in the healthcare experience. It would serve as a shared foundation for innovation, efficiency, and better outcomes across the entire healthcare ecosystem.

VB-1.	What incentives could encourage APMs such as accountable care organizations (ACOs) or participants in Medicare Shared Savings Program (MSSP) to leverage digital health management and care navigation products more often and more effectively with their patients? What are the current obstacles preventing broader digital product adoption for patients in ACOs?	ACOs and MSSP participants are well-positioned to benefit from digital health and care navigation tools, but adoption has been limited by several persistent barriers, chief among them being a lack of trusted, accurate provider data. Without reliable infrastructure to identify which providers are in-network, which locations they practice at, and what services they offer, care navigation tools fall short of their promise. This undermines both provider and patient trust, and leads to missed opportunities for proactive, coordinated care. Current obstacles include: Inaccurate or fragmented provider directories, which make it difficult to guide patients to the right provider or facility Limited integration between digital tools and core systems, such as EHRs, scheduling platforms, and claims data Administrative burden, particularly for smaller ACOs, to validate and maintain the provider data needed to power these tools effectively Certify addresses these challenges at the infrastructure level by delivering a national, real-time, continuously verified provider data platform, available via Open API. This foundation allows care navigation and health management tools to integrate directly into our data, and to operate with clarity and confidence, guiding patients to the right provider, at the right time, in the right setting. To encourage broader and more effective use of digital tools by ACOs and MSSP participants, CMS could consider the following incentives: Incentivize use of certified provider infrastructure (such as Certify) to power care navigation and engagement tools, ensuring the accuracy and utility of patient-facing applications Include digital engagement and data quality as a performance metric in MSSP scoring or shared savings formulas Provide technical and financial support for smaller or rural ACOs to integrate with national infrastructure and deploy patient-friendly tools

		Tie bonus payments or regulatory relief to digital tool utilization, especially when tools are used to address high-cost, high-need populations Incentivize provider engagement in data maintenance, for example by rewarding accurate, up-to-date information that supports patient navigation and reduces network leakage By removing the burden of maintaining provider data from individual ACOs and instead offering infrastructure-level solutions like Certify, CMS can unlock a more consistent and effective digital experience for all patients in value-based arrangements. This not only supports care coordination and cost savings, but helps make digital health tools a natural, trusted part of the care journey.
VB-4	What are the essential data types needed for successful participation in value-based care arrangements?	Successful value-based care (VBC) arrangements require a broad ecosystem of accurate, actionable data. While much of the focus is often on clinical outcomes and cost data, these models fundamentally rely on one foundational layer: accurate, real-time provider data. Without it, patient attribution breaks down, quality measurement is flawed, care coordination falters, and incentives become misaligned. From Certify's perspective as the provider data infrastructure powering the next generation of digital health tools and operations, the following data types are essential to value-based care success: Verified provider identity and credentials - Accurate NPI, taxonomy, licensure, and board certifications are necessary to validate that the provider is qualified and eligible to participate in a given model. This data also supports downstream integration with credentialing, contracting, and compliance systems. Provider affiliations and practice locations - Understanding which groups, facilities, and networks a provider is affiliated with, and at which locations they actively see patients, is critical for accurate patient attribution, care coordination, and reimbursement under value-based arrangements. Network participation data - Knowing which health plans and products a provider participates in, and under what terms, ensures patients are routed correctly and that attribution models reflect the real-world delivery landscape. Demographic and cultural competency attributes -

		Language proficiency, ethnicity, and cultural training or focus areas enable more personalized and equitable patient-provider matching, which is especially important for closing gaps in care and addressing social determinants of health under VBC models. Historical data lineage and change tracking - To support auditability, compliance, and performance tracking, value-based care programs need visibility into when and how provider data changes, across systems and over time. Certify's infrastructure provides this through automated delta tracking, survivorship logic, and end-to-end data lineage. When these provider data elements are made available through a real-time, API-driven infrastructure like Certify, they serve as the backbone for a wide range of value-based workflows—including accurate patient attribution, risk adjustment, network adequacy validation, quality measure assignment, and care navigation. In short, clinical and claims data may power the engine of value-based care, but it's accurate provider data that lays the track. Without it, VBC programs are built on unreliable foundations. Certify is purpose-built to ensure this foundation is solid, scalable, and accessible across the healthcare ecosystem.